

Introduction

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This is the third edition of the *Diagnostic and Statistical Manual of Mental Disorders* of the American Psychiatric Association, better known simply as DSM-III. The development of this manual over the last five years has not gone unnoticed; in fact, it is remarkable how much interest (alarm, despair, excitement, joy) has been shown in successive drafts of this document. The reasons for this interest are many.

First of all, over the last decade there has been growing recognition of the importance of diagnosis for both clinical practice and research. Clinicians and research investigators must have a common language with which to communicate about the disorders for which they have professional responsibility. Planning a treatment program must begin with an accurate diagnostic assessment. The efficacy of various treatment modalities can be compared only if patient groups are described using diagnostic terms that are clearly defined.

Secondly, from its very beginning, drafts of DSM-III have been widely circulated for critical review and use by clinicians and investigators. This made them aware of the many fundamental ways in which DSM-III differs from its predecessor, DSM-II, and from its international contemporary, the mental disorders chapter of the ninth revision of the *International Classification of Diseases* (ICD-9). For example, DSM-III includes such new features as diagnostic criteria, a multiaxial approach to evaluation, much-expanded descriptions of the disorders and many additional categories (some with newly-coined names); and it does not include several time-honored categories.

Finally, interest in the development of this manual is due to awareness that DSM-III reflects an increased commitment in our field to reliance on data as the basis for understanding mental disorders.

BACKGROUND*

The first edition of the American Psychiatric Association's *Diagnostic and Statistical Manual of Mental Disorders* appeared in 1952. This was the first official manual of mental disorders to contain a glossary of descriptions of the diagnostic categories. The use of the term "reaction" throughout the classification reflected the influence of Adolf Meyer's psychobiological view that mental disorders represented reactions of the personality to psychological, social, and biological factors. In the development of the second edition (DSM-II), a decision was made to base the classification on the mental disorders section of the eighth revision of the *International Classification of Diseases*, for which representatives of the American Psychiatric Association had provided consultation. Both DSM-II and

* Some readers may wish, for now, to skip *Background* and *The Process of Development of DSM-III* and plunge directly into *Basic Concepts* on p.5.

ICD-8 went into effect in 1968. The DSM-II classification did not use the term "reaction" and used diagnostic terms that by and large did not imply a particular theoretical framework for understanding the nonorganic mental disorders.

In 1974 the American Psychiatric Association, through its Council on Research and Development, appointed a Task Force on Nomenclature and Statistics to begin work on the development of DSM-III, recognizing that ICD-9 was scheduled to go into effect in January 1979. By the time this new Task Force was constituted, the mental disorders section of ICD-9, which included its own glossary, was nearly completed. Although representatives of the American Psychiatric Association had worked closely with the World Health Organization in the development of ICD-9, there was some concern that the ICD-9 classification and glossary would not be suitable for use in the United States. Most importantly, many specific areas of the classification did not seem sufficiently detailed for clinical and research use. For example, the ICD-9 classification contains only one category for "frigidity and impotence"—despite the substantial work in the area of psychosexual dysfunctions that has identified several specific types with different clinical pictures and treatment implications. In addition, the glossary of ICD-9 was believed by many to be less than optimal in that it had not made use of such recent major methodological developments as specified diagnostic criteria and the multiaxial approach to evaluation.

For these reasons the Task Force was directed to prepare a new classification and glossary that would, as much as possible, reflect the most current state of knowledge regarding mental disorders while maintaining compatibility with ICD-9. Like its predecessors, DSM-I and DSM-II, DSM-III had to be, first of all, clinically useful, while also providing a basis for research and administrative use.

The Task Force. Task Force members, and consultants from the fields of psychology and epidemiology, were selected because of their special interest in various aspects of diagnosis. Most had made significant contributions to the literature on diagnosis. As the work progressed, additional members were added to ensure representation of different perspectives and areas of expertise.

From the beginning, the Task Force functioned as a steering committee to oversee the ongoing work. All of its members shared a commitment to the attainment in DSM-III of the following goals:

- clinical usefulness for making treatment and management decisions in varied clinical settings;
- reliability of the diagnostic categories;
- acceptability to clinicians and researchers of varying theoretical orientations;
- usefulness for educating health professionals;
- maintaining compatibility with ICD-9, except when departures are unavoidable;
- avoiding the introduction of new terminology and concepts that break with tradition, except when clearly needed;
- reaching consensus on the meaning of necessary diagnostic terms that have been used inconsistently, and avoiding the use of terms that have outlived their usefulness;
- consistency with data from research studies bearing on the validity of diagnostic categories;

- suitability for describing subjects in research studies;
- being responsive during the development of DSM-III to critiques by clinicians and researchers.

The major job of the Task Force has been to determine the most effective strategies for ensuring that the final document attained each goal to as great an extent as possible without compromising the other goals. Thus, the Task Force evaluated all proposals for changes in DSM-III that might affect the attainment of these goals. These proposals came from members of the Task Force, advisory committees, liaison committees with professional organizations, and participants in the DSM-III Field Trials. Finally, the Task Force reviewed drafts of the text and diagnostic criteria.

In attempting to resolve various diagnostic issues, the Task Force relied, as much as possible, on research evidence relevant to various kinds of diagnostic validity. For example, when discussing a problematic diagnostic category, the Task Force considered how the disorder, if defined as proposed, provided information relevant to treatment planning, course, and familial pattern. It should come as no surprise to the reader that even when data were available from relevant research studies, Task Force members often differed in their interpretations of the findings.

Advisory Committees and Other Consultants. Successive drafts of DSM-III were prepared by fourteen advisory committees composed of individuals with special expertise in each substantive area. In addition, a group of consultants provided advice and information on a variety of special areas.

Council on Research and Development. This component of the American Psychiatric Association appointed the Task Force and regularly reviewed progress being made in the development of DSM-III. In addition, in the fall of 1978 the Council held an all-day meeting at which some APA members voiced concerns about certain aspects of DSM-III. After reviewing these concerns, the Council approved the Task Force's approach to solutions of the problems that had been raised.

Assembly Liaison Committee. In early 1976, the APA Assembly, composed of representatives from all of the APA's district branches, appointed a Liaison Committee to review the development of DSM-III and to report regularly to the Assembly. This committee received correspondence on major issues, reviewed successive drafts of DSM-III, and met a number of times with the chairperson of the Task Force. On several occasions the Assembly Liaison Committee arranged for the chairperson of the Task Force to discuss a particular controversial issue with the entire Assembly. The Assembly Liaison Committee was invaluable in articulating the concerns of the membership of the APA, which is composed largely of clinicians whose primary professional activity is patient care.

Other Components of the APA. The chairperson of the Task Force reported on several occasions to the Reference Committee and the Board of Trustees on specific issues of concern. In addition, in April 1979, a meeting was held with an

Ad Hoc Committee on DSM-III of the Board of Trustees to review specific concerns about DSM-III that had been expressed by members of the APA. Other components of the APA, such as the Committee on Confidentiality and the Committee on Women, also reviewed DSM-III from their own perspectives as it was being developed.

Liaison with Other Professional Organizations. The following groups that were particularly interested in the development of DSM-III established liaison committees with the Task Force: the Academy of Psychiatry and the Law, the American Academy of Child Psychiatry, the American Academy of Psychoanalysis, the American Association of Chairmen of Departments of Psychiatry, the American College Health Association, the American Orthopsychiatric Association, the American Psychoanalytic Association, and the American Psychological Association. These committees received drafts of DSM-III and were invited to make comments and suggestions and to express their concerns. In most instances, differences in points of view between a liaison committee and the Task Force were resolved to the satisfaction of all concerned. When this was not possible and differences were left unresolved, the issues were at least clarified.

THE PROCESS OF DEVELOPMENT OF DSM-III

In May 1975, at a special session of the Annual Meeting of the APA, an initial draft of the DSM-III classification was presented. At each subsequent Annual Meeting a special session was held on some aspect of DSM-III. In addition, a special conference was held in St. Louis, Missouri, in June 1976, to examine "DSM-III in Midstream." This conference, co-sponsored by the Missouri Institute of Psychiatry and the American Psychiatric Association, was attended by approximately 100 professionals with expertise or special interests in various aspects of DSM-III, most of whom had previously had no direct involvement in the development of DSM-III. As a result of discussions at this conference, additional diagnostic categories were added, some were deleted, and a decision was made to proceed with the development of the multiaxial system.

The DSM-III classification and the rationale for the strategies used in its development have been presented throughout the past four years at local, national, and international professional meetings. In addition, the 4/15/77 draft and successive drafts of DSM-III have been available to the profession for critical review. Throughout this period there has been continual consideration of various solutions to difficult diagnostic problems, often based on summaries of actual cases submitted to the Task Force from all quarters. Whenever possible, attempts have been made to seek the advice of experts in each specific area under consideration.

Field Trials. In the past, new classifications of mental disorders have not been extensively subjected to clinical trials before official adoption. The Task Force believed that field trials using drafts of DSM-III should be conducted during the development process to identify problem areas in the classification and to try out solutions to these problems. In addition, because of the many proposed changes in the classification, it was important to demonstrate its clini-

cal acceptability and usefulness in a variety of settings by clinicians of varying theoretical orientations.

For these reasons, a series of field trials was conducted, beginning in 1977 and culminating in a two year NIMH-sponsored field trial from September 1977 to September 1979. In all, 12,667 patients were evaluated by approximately 550 clinicians, 474 of whom were in 212 different facilities, using successive drafts of DSM-III. Critiques of all portions of DSM-III by the field trial participants resulted in numerous changes, as did reviews of case summaries submitted by those participants. Frequently, participants completed questionnaires regarding specific diagnostic issues and their attitudes toward DSM-III and its innovative features. The results indicated that the great majority of participants, regardless of theoretical orientations, had a favorable response to DSM-III.

Perhaps the most important part of the study was the evaluation of diagnostic reliability by having pairs of clinicians make independent diagnostic judgments of several hundred patients. The results, which are presented in an appendix, generally indicate far greater reliability than had previously been obtained with DSM-II.

ICD-9-CM. Because of dissatisfaction with ICD-9 expressed by organizations representing subspecialties of medicine (not including the American Psychiatric Association), a decision was made to modify the ICD-9 for use in the United States by expanding the four-digit ICD-9 codes to five-digit ICD-9-CM (for clinical modification) codes whenever greater specificity was required. This modification was prepared for the United States National Center for Health Statistics by the Council on Clinical Classifications. The American Psychiatric Association, in December 1976, was invited to submit recommendations for alternate names and additional categories based on subdivisions of already existing ICD-9 categories. This made it possible for the developing DSM-III classification and its diagnostic terms to be included in the ICD-9-CM classification, which in January 1979 became the official system in this country for recording all "diseases, injuries, impairments, symptoms, and causes of death." The ICD-9-CM codes and diagnostic terms for mental disorders are included in Appendix D.

Many ICD-9-CM codes and terms are not included in the DSM-III classification. However, these are generally acceptable to third party payers and most record-keeping systems.

Final Approval. In May 1979, at the Annual Meeting of the APA in Chicago, the Assembly and the Council on Research and Development formally approved the final draft of DSM-III. In June, it was approved by the Reference Committee and the Board of Trustees.

BASIC CONCEPTS

Mental Disorder. Although this manual provides a classification of mental disorders, there is no satisfactory definition that specifies precise boundaries for the concept "mental disorder" (also true for such concepts as physical disorder and

mental and physical health). Nevertheless, it is useful to present concepts that have influenced the decision to include certain conditions in DSM-III as mental disorders and to exclude others.

In DSM-III each of the mental disorders is conceptualized as a clinically significant behavioral or psychological syndrome or pattern that occurs in an individual and that is typically associated with either a painful symptom (distress) or impairment in one or more important areas of functioning (disability). In addition, there is an inference that there is a behavioral, psychological, or biological dysfunction, and that the disturbance is not only in the relationship between the individual and society. (When the disturbance is *limited* to a conflict between an individual and society, this may represent social deviance, which may or may not be commendable, but is not by itself a mental disorder.)

In DSM-III there is no assumption that each mental disorder is a discrete entity with sharp boundaries (discontinuity) between it and other mental disorders, as well as between it and No Mental Disorder. For example, there has been a continuing controversy as to whether or not severe depressive disorder and mild depressive disorder differ from each other qualitatively (discontinuity between diagnostic entities) or quantitatively (a difference on a severity continuum). The inclusion of Major Depression With and Without Melancholia as separate categories in DSM-III is justified by the clinical usefulness of the distinction. This does not imply a resolution of the controversy as to whether or not these conditions are in fact quantitatively or qualitatively different.

A common misconception is that a classification of mental disorders classifies individuals, when actually what are being classified are disorders that individuals have. For this reason, the text of DSM-III avoids the use of such phrases as "a schizophrenic" or "an alcoholic," and instead uses the more accurate, but admittedly more wordy "an individual with Schizophrenia" or "an individual with Alcohol Dependence."

Another misconception is that all individuals described as having the same mental disorder are alike in all important ways. Although all the individuals described as having the same mental disorder show at least the defining features of the disorder, they may well differ in other important ways that may affect clinical management and outcome.

Conditions Not Attributable to a Mental Disorder. In DSM-III it is recognized that a behavioral or psychological problem may appropriately be a focus of professional attention or treatment even though it is not attributable to a mental disorder. A limited listing of codes, taken from the V codes section of ICD-9-CM, is provided for noting such problems.

Descriptive Approach. For some of the mental disorders, the etiology or pathophysiological processes are known. For example, in the Organic Mental Disorders, organic factors necessary for the development of the disorders have been identified or are presumed. Another example is Adjustment Disorder, in which the disturbance is a reaction to psychosocial stress.

For most of the DSM-III disorders, however, the etiology is unknown. A variety of theories have been advanced, buttressed by evidence—not always

convincing—to explain how these disorders come about. The approach taken in DSM-III is atheoretical with regard to etiology or pathophysiological process except for those disorders for which this is well established and therefore included in the definition of the disorder. Undoubtedly, with time, some of the disorders of unknown etiology will be found to have specific biological etiologies, others to have specific psychological causes, and still others to result mainly from a particular interplay of psychological, social and biological factors.

The major justification for the generally atheoretical approach taken in DSM-III with regard to etiology is that the inclusion of etiological theories would be an obstacle to use of the manual by clinicians of varying theoretical orientations, since it would not be possible to present all reasonable etiological theories for each disorder. For example, Phobic Disorders are believed by many to represent a displacement of anxiety resulting from the breakdown of defensive operations for keeping internal conflict out of consciousness. Other investigators explain phobias on the basis of learned avoidance responses to conditioned anxiety. Still others believe that certain phobias result from a dysregulation of basic biological systems mediating separation anxiety. In any case, as the field trials have demonstrated, clinicians can agree on the identification of mental disorders on the basis of their clinical manifestations without agreeing on how the disturbances come about.

Because DSM-III is generally atheoretical with regard to etiology, it attempts to describe comprehensively what the manifestations of the mental disorders are, and only rarely attempts to account for *how* the disturbances come about, unless the mechanism is included in the definition of the disorder. This approach can be said to be “descriptive” in that the definitions of the disorders generally consist of descriptions of the clinical features of the disorders. These features are described at the lowest order of inference necessary to describe the characteristic features of the disorder. Frequently the order of inference is relatively low, and the characteristic features consist of easily identifiable behavioral signs or symptoms, such as disorientation, mood disturbance, or psychomotor agitation. For some disorders, however, particularly the Personality Disorders, a much higher order of inference is necessary. For example, one of the criteria for Borderline Personality Disorder is “identity disturbance manifested by uncertainty about several issues relating to identity, such as self-image, gender identity, long-term goals or career choice, friendship patterns, values and loyalties.”

This descriptive approach is also used in the division of the mental disorders into diagnostic classes. All of the disorders without known etiology or pathophysiological process are grouped together on the basis of shared clinical features.

The subdivision of each diagnostic class into specific disorders, with even further subdivision in some cases, reflects the best judgment of the Task Force and its Advisory Committees that such subdivision will be useful. In this regard we have been guided by the judgments of those clinicians who will be making most use of each portion of the classification. For example, the subdivision of Psychosexual Dysfunctions into seven specific disorders is in response to the expressed needs of clinicians who specialize in the treatment of these conditions. (It soon became apparent that the criticism that a subdivision in a particular area of the classification was useless always came from clinicians who specialized

in other areas.) It should be noted, however, that the judgments of clinicians concerning the necessity for including new categories were not accepted uncritically. Although initially many new categories were added in an effort to be inclusive, experience in the field trials and lack of validity evidence from the literature resulted in the elimination of several proposed categories.

Diagnostic Criteria. Since in DSM-I, DSM-II, and ICD-9 explicit criteria are not provided, the clinician is largely on his or her own in defining the content and boundaries of the diagnostic categories. In contrast, DSM-III provides specific diagnostic criteria as guides for making each diagnosis since such criteria enhance interjudge diagnostic reliability. It should be understood, however, that for most of the categories the diagnostic criteria are based on clinical judgment, and have not yet been fully validated by data about such important correlates as clinical course, outcome, family history, and treatment response. Undoubtedly, with further study the criteria for many of the categories will be revised.

Multiaxial Evaluation. DSM-III recommends the use of a multiaxial system for evaluation to ensure that certain information that may be of value in planning treatment and predicting outcome for each individual is recorded on each of five axes, the first three of which constitute an official diagnostic evaluation.

Axes I and II include all of the mental disorders. (Two classes of mental disorders, Personality Disorders and Specific Developmental Disorders, are assigned to Axis II, whereas all of the other mental disorders are assigned to Axis I. The reason for this is discussed on p. 23. This does not imply that these Axis II disorders are not mental disorders.)

Axis III is for physical disorders and conditions. The separation of this axis from the mental disorders axes, is based on the tradition of separating those disorders whose manifestations are primarily behavioral or psychological (i.e., mental disorders) from those whose manifestations are not. It is necessary to have a term that can be applied to all of the disorders that are not considered "mental disorders." The phrase "organic disorder" would incorrectly imply the absence of physical factors in "mental" disorders. Hence, this manual uses the term "physical disorder," recognizing that the boundaries for these two classes of disorders ("mental" and "physical" disorders) change as our understanding of the pathophysiology of these disorders increases.

Axis IV, Severity of Psychosocial Stressors and Axis V, Highest Level of Adaptive Functioning Past Year, are for use in special clinical or research settings and provide information additional to the official DSM-III diagnoses (Axes I, II, and III) that is of value for treatment planning and predicting outcome.

Hierarchical Organization of Diagnostic Classes. In some mental disorders, for example, Organic Mental Disorders, there is a wide range of signs and symptoms. In others, such as Anxiety Disorders, only a limited range of signs and symptoms is seen. For this reason, the order in which diagnostic classes are listed represents, to some extent, a hierarchy in which a disorder high in the hierarchy may have features found in disorders lower in the hierarchy, but

not the reverse. This hierarchical relationship makes it possible to present the differential diagnosis of major symptom areas in a series of decision trees (see Appendix A).

Systematic Description. The text of DSM-III systematically describes each disorder in terms of current knowledge in the following areas: essential features, associated features, age at onset, course, impairment, complications, predisposing factors, prevalence, sex ratio, familial pattern, and differential diagnosis. Although descriptively comprehensive, DSM-III is not a textbook, since it does not include information about theories of etiology, management and treatment. It should also be noted that the DSM-III classification of mental disorders does not attempt to classify disturbed dyadic, family, or other interpersonal relationships.

Glossary of Technical Terms. Technical terms used in the text for describing the disorders are defined in a glossary in Appendix B.

Annotated Comparative Listing of DSM-II and DSM-III. The profession is entitled to know the rationale for all of the major changes that have resulted in the DSM-III classification of mental disorders. For this reason, included in Appendix C is a table containing an explanation for each major change made and new category added, with references from the scientific literature. With the use of this table, the reader can more easily make the transition from the DSM-II to the DSM-III classification and understand the reasons for the changes.

NEUROTIC DISORDERS

Throughout the development of DSM-III the omission of the DSM-II diagnostic class of Neuroses has been a matter of great concern to many clinicians, and requires an explanation.

When Freud first used the term "psychoneurosis," he was referring to only four subtypes: anxiety neurosis, anxiety hysteria (phobia), obsessive compulsive neurosis, and hysteria. Freud used the term both *descriptively* (to indicate a painful symptom in an individual with intact reality testing) and to indicate the *etiological process* (unconscious conflict arousing anxiety and leading to the maladaptive use of defensive mechanisms that result in symptom formation).

At the present time, however, there is no consensus in our field as to how to define "neurosis." Some clinicians limit the term to its descriptive meaning whereas others also include the concept of a specific etiological process. To avoid ambiguity, the term *neurotic disorder* should be used only descriptively. This is consistent with the use of this term in ICD-9. The term *neurotic process*, on the other hand, should be used when the clinician wishes to indicate the concept of a specific etiological process involving the following sequence: unconscious conflicts between opposing wishes or between wishes and prohibitions, which causes unconscious perception of anticipated danger or dysphoria, which leads to use of defense mechanisms that result in either symptoms, personality disturbance, or both.

The term *neurotic disorder* thus refers to a mental disorder in which the predominant disturbance is a symptom or group of symptoms that is distressing

to the individual and is recognized by him or her as unacceptable and alien (ego-dystonic); reality testing is grossly intact; behavior does not actively violate gross social norms (although functioning may be markedly impaired); the disturbance is relatively enduring or recurrent without treatment and is not limited to a transitory reaction to stressors; and there is no demonstrable organic etiology or factor.

Although many psychodynamically-oriented clinicians believe that the neurotic process always plays a central role in the development of neurotic disorders, there are other theories about how these disorders develop. For example, there are social learning, cognitive, behavioral, and biological models that attempt to explain the development of various neurotic disorders.

Thus, the term *neurotic disorder* is used in DSM-III without any implication of a special etiological process. Neurotic disorder, defined descriptively, is roughly equivalent to the psychoanalytic concept of "symptom neurosis." (This is distinguished from "character neurosis" which is roughly equivalent to the DSM-III concept of Personality Disorder. According to modern psychoanalytic theory, the neurotic process is involved in the development of both symptom neuroses and character neuroses.)

In DSM-III the Neurotic Disorders are included in Affective, Anxiety, Somatoform, Dissociative, and Psychosexual Disorders. These diagnostic classes are listed together in the DSM-III classification to facilitate the location of Neurotic Disorders. Preceding the listing of the class of Affective Disorders is a statement indicating that Neurotic Disorders are included in these five DSM-III classes.

It should be noted that the ICD-9 category Neurotic Disorders, also defined descriptively, includes only those categories that historically have been included as "neuroses" in previous standard classifications. These previous classifications did not contain some of the DSM-III categories, such as Psychosexual Disorders, that unquestionably include some disorders falling within the concept of Neurotic Disorders.

Alternative approaches to the issue of the relationship of Neurotic Disorders to the DSM-III classification were considered. If the DSM-III classification had included a category of Neurotic Disorders that was limited to those disorders included in the ICD-9 category, the potential value of the term Neurotic Disorder would have been limited by a lack of adherence to its descriptive meaning. On the other hand, to have grouped together all of the specific DSM-III categories that are usually considered to be Neurotic Disorders would have required separating some Affective Disorders from the other Affective Disorders, some Psychosexual Disorders from the other Psychosexual Disorders, and some Dissociative Disorders from other members of that class. The possible advantages of this approach seemed to be far outweighed by the disadvantage of fragmenting several diagnostic classes. Similarly, it was judged unwise to group all psychotic disorders together, as is done in ICD-9.

USING DSM-III

The major justification for the generally atheoretical approach taken in Several features are included that can help the user become adept at making

optimal use of the manual. By examining the listing of Axis I and Axis II diagnoses and conditions contained in Chapter 1, the user can become familiar with the organization of the classification into major and minor diagnostic classes. By studying Chapter 2, *The Use of This Manual*, the reader will learn how to use the multiaxial system, record principal and secondary diagnoses, indicate various levels of diagnostic certainty, and use the diagnostic criteria as guides in making diagnoses. Chapter 3 contains the text and criteria for all of the diagnostic categories. The user will want to pay particular attention to those sections that are most appropriate to the kind of clinical or research work that he or she does.

In making a DSM-III diagnosis the clinician may find it more convenient to consult the *Quick Reference to the Diagnostic Criteria from DSM-III*, (Mini-D), a pocket-sized booklet sold separately, that contains only the classification, the diagnostic criteria, a listing of the most important conditions to be considered in a differential diagnosis of each category, and an index. It should be noted that the index in both this book and the *Quick Reference* can be used when the clinician is in doubt about the DSM-III term that corresponds to a DSM-II term or to the name of some other widely used diagnostic category.

EVALUATION FOR TREATMENT PLANNING

Making a DSM-III diagnosis represents an initial step in a comprehensive evaluation leading to the formulation of a treatment plan. Additional information about the individual being evaluated beyond that required to make a DSM-III diagnosis will invariably be necessary.

For instance, the clinician considering a psychodynamically-oriented treatment will pay particular attention to the nature of the interaction of the patient with the clinician during the interview, focusing on the particular way the patient molds and distorts the interview situation in order to make it conform to his or her deeply ingrained (usually unconscious) fantasies, attitudes, and expectations about interpersonal relationships. The nature of these transference phenomena will be noted in order to predict future behavior in the treatment setting and to shed light on the patient's early developmental experiences and the conflicts that underlie the current disturbance. The clinician will note the patient's ability to reflect upon feelings and fantasies as they are being experienced. The clinician will also monitor his or her own responses to the patient as an indicator of the patient's unconscious conflicts and defensive style. Finally, the clinician will make a psychodynamic diagnostic formulation that is an explanation of the patient's psychopathology in terms of the nature of the unconscious conflicts and defense mechanisms, and the origins of the current behavior in early life experience.

The clinician considering behavior therapy will do a functional analysis of the behavior disturbance. This begins by defining the problem behavior as objectively as possible in terms of developmental history and present antecedents and consequences. These may be external (environmental, social) or internal (affects, cognitions). When appropriate, attention will be paid to the patient's idiosyncratic thinking patterns (cognitions) and unfounded beliefs

about himself or herself and his or her relationship to others (schemata) which may contribute to the onset or maintenance of the problem behavior. The frequency of the problem behavior and the circumstances under which it occurs are monitored during the behavioral analysis and as treatment progresses. The functional analysis leads to the formulation of a set of hypotheses concerning the acquisition and maintenance of the problem behavior, which is then tested by the application of a specific behavioral treatment.

A clinician considering family therapy will need information about how the presenting problem affects the "identified patient" and the other family members as individuals and as a social unit, how the family members relate to each other, and how they could more effectively provide mutual support in dealing with current and future problems. In addition, the clinician will want to know how the family fits into the broader social network, which includes the therapist and other health-care providers, and how the family can make most effective use of these resources.

The clinician considering somatic therapy will pay particular attention to how any abnormalities detected during a medical examination will affect the choice of a somatic therapy. If the patient is currently on a psychoactive medication and is not responding satisfactorily, it may be useful to clarify the diagnosis and treatment needs of the patient by observing the patient without medication, making sure that this is done in circumstances that protect the patient's welfare. The patient's response to previous somatic therapy and its adequacy in terms of choice, dosage, and duration will be reviewed. The patient's attitude toward somatic treatment will be explored; and when necessary, an attempt will be made to relieve unrealistic anxieties about such treatment.

CAUTIONS

The purpose of DSM-III is to provide clear descriptions of diagnostic categories in order to enable clinicians and investigators to diagnose, communicate about, study, and treat various mental disorders. The use of this manual for non-clinical purposes, such as determination of legal responsibility, competency or insanity, or justification for third-party payment, must be critically examined in each instance within the appropriate institutional context.

THE FUTURE

In the several years that it has taken to develop DSM-III, there have been several instances when major changes in initial drafts were necessary because of new findings. Thus, this final version of DSM-III is only one still frame in the ongoing process of attempting to better understand mental disorders.

Chapter One

The DSM-III Classification

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DSM-III CLASSIFICATION: AXES I AND II CATEGORIES AND CODES

All official DSM-III codes and terms are included in ICD-9-CM. However, in order to differentiate those DSM-III categories that use the same ICD-9-CM codes, unofficial non-ICD-9-CM codes are provided in parentheses for use when greater specificity is necessary.

The long dashes indicate the need for a fifth-digit subtype or other qualifying term.

DISORDERS USUALLY FIRST EVIDENT IN INFANCY, CHILDHOOD OR ADOLESCENCE

Mental retardation

(Code in fifth digit: 1 = with other behavioral symptoms [requiring attention or treatment and that are not part of another disorder], 0 = without other behavioral symptoms.)

317.0(x) Mild mental retardation, _____

318.0(x) Moderate mental retardation, _____

318.1(x) Severe mental retardation, _____

318.2(x) Profound mental retardation, _____

319.0(x) Unspecified mental retardation, _____

Attention deficit disorder

314.01 with hyperactivity

314.00 without hyperactivity

314.80 residual type

Conduct disorder

312.00 undersocialized, aggressive

312.10 undersocialized, nonaggressive

312.23 socialized, aggressive

312.21 socialized, nonaggressive

312.90 atypical

Anxiety disorders of childhood or adolescence

309.21 Separation anxiety disorder

313.21 Avoidant disorder of childhood or adolescence

313.00 Overanxious disorder

Other disorders of infancy, childhood or adolescence

313.89 Reactive attachment disorder of infancy

313.22 Schizoid disorder of childhood or adolescence

313.23 Elective mutism

313.81 Oppositional disorder

313.82 Identity disorder

Eating disorders

307.10 Anorexia nervosa

307.51 Bulimia

307.52 Pica

307.53 Rumination disorder of infancy

307.50 Atypical eating disorder

Stereotyped movement disorders

307.21 Transient tic disorder

307.22 Chronic motor tic disorder

307.23 Tourette's disorder

307.20 Atypical tic disorder

307.30 Atypical stereotyped movement disorder

Other disorders with physical manifestations

307.00 Stuttering

307.60 Functional enuresis

307.70 Functional encopresis

307.46 Sleepwalking disorder

307.46 Sleep terror disorder (307.49)

Pervasive developmental disorders

Code in fifth digit: 0 = full syndrome present, 1 = residual state.

299.0x Infantile autism, _____

299.8x Childhood onset pervasive developmental disorder, _____

299.8x Atypical, _____

Specific developmental disorders

Note: These are coded on Axis II.

315.00 Developmental reading disorder

315.10 Developmental arithmetic disorder

315.31 Developmental language disorder

315.39 Developmental articulation disorder

315.50 Mixed specific developmental disorder

315.90 Atypical specific developmental disorder

ORGANIC MENTAL DISORDERS

Section 1. Organic mental disorders whose etiology or pathophysiological process is listed below (taken from the mental disorders section of ICD-9-CM).

Dementias arising in the senium and presenium

Primary degenerative dementia, senile onset,

- 290.30 with delirium
- 290.20 with delusions
- 290.21 with depression
- 290.00 uncomplicated

Code in fifth digit:

1 = with delirium, 2 = with delusions, 3 = with depression, 0 = uncomplicated.

- 290.1x Primary degenerative dementia, presenile onset, _____
- 290.4x Multi-infarct dementia, _____

Substance-induced

Alcohol

- 303.00 intoxication
- 291.40 idiosyncratic intoxication
- 291.80 withdrawal
- 291.00 withdrawal delirium
- 291.30 hallucinosis
- 291.10 amnestic disorder

Code severity of dementia in fifth digit: 1 = mild, 2 = moderate, 3 = severe, 0 = unspecified.

- 291.2x Dementia associated with alcoholism, _____

Barbiturate or similarly acting sedative or hypnotic

- 305.40 intoxication (327.00)
- 292.00 withdrawal (327.01)
- 292.00 withdrawal delirium (327.02)
- 292.83 amnestic disorder (327.04)

Opioid

- 305.50 intoxication (327.10)
- 292.00 withdrawal (327.11)

Cocaine

- 305.60 intoxication (327.20)

Amphetamine or similarly acting sympathomimetic

- 305.70 intoxication (327.30)
- 292.81 delirium (327.32)

- 292.11 delusional disorder (327.35)
- 292.00 withdrawal (327.31)

Phencyclidine (PCP) or similarly acting arylcyclohexylamine

- 305.90 intoxication (327.40)
- 292.81 delirium (327.42)
- 292.90 mixed organic mental disorder (327.49)

Hallucinogen

- 305.30 hallucinosis (327.56)
- 292.11 delusional disorder (327.55)
- 292.84 affective disorder (327.57)

Cannabis

- 305.20 intoxication (327.60)
- 292.11 delusional disorder (327.65)

Tobacco

- 292.00 withdrawal (327.71)

Caffeine

- 305.90 intoxication (327.80)

Other or unspecified substance

- 305.90 intoxication (327.90)
- 292.00 withdrawal (327.91)
- 292.81 delirium (327.92)
- 292.82 dementia (327.93)
- 292.83 amnestic disorder (327.94)
- 292.11 delusional disorder (327.95)
- 292.12 hallucinosis (327.96)
- 292.84 affective disorder (327.97)
- 292.89 personality disorder (327.98)
- 292.90 atypical or mixed organic mental disorder (327.99)

Section 2. Organic brain syndromes whose etiology or pathophysiological process is either noted as an additional diagnosis from outside the mental disorders section of ICD-9-CM or is unknown.

- 293.00 Delirium
- 294.10 Dementia
- 294.00 Amnestic syndrome
- 293.81 Organic delusional syndrome
- 293.82 Organic hallucinosis
- 293.83 Organic affective syndrome
- 310.10 Organic personality syndrome
- 294.80 Atypical or mixed organic brain syndrome

SUBSTANCE USE DISORDERS

Code in fifth digit: 1 = continuous,
2 = episodic, 3 = in remission, 0 =
unspecified.

- 305.0x Alcohol abuse, _____
- 303.9x Alcohol dependence
(Alcoholism), _____
- 305.4x Barbiturate or similarly acting
sedative or hypnotic abuse,
- 304.1x Barbiturate or similarly acting
sedative or hypnotic de-
pendence, _____
- 305.5x Opioid abuse, _____
- 304.0x Opioid dependence, _____
- 305.6x Cocaine abuse, _____
- 305.7x Amphetamine or similarly
acting sympathomimetic
abuse, _____
- 304.4x Amphetamine or similarly
acting sympathomimetic
dependence, _____
- 305.9x Phencyclidine (PCP) or similarly
acting arylcyclohexylamine
abuse, _____ (328.4x)
- 305.3x Hallucinogen abuse, _____
- 305.2x Cannabis abuse, _____
- 304.3x Cannabis dependence, _____
- 305.1x Tobacco dependence, _____
- 305.9x Other, mixed or unspecified
substance abuse, _____
- 304.6x Other specified substance
dependence, _____
- 304.9x Unspecified substance
dependence, _____
- 304.7x Dependence on combination
of opioid and other non-
alcoholic substance, _____
- 304.8x Dependence on combination of
substances, excluding opioids
and alcohol, _____

SCHIZOPHRENIC DISORDERS

Code in fifth digit: 1 = subchronic,
2 = chronic, 3 = subchronic with acute
exacerbation, 4 = chronic with acute
exacerbation, 5 = in remission, 0 =
unspecified.

- Schizophrenia,
- 295.1x disorganized, _____
 - 295.2x catatonic, _____
 - 295.3x paranoid, _____
 - 295.9x undifferentiated, _____
 - 295.6x residual, _____

PARANOID DISORDERS

- 297.10 Paranoia
- 297.30 Shared paranoid disorder
- 298.30 Acute paranoid disorder
- 297.90 Atypical paranoid disorder

**PSYCHOTIC DISORDERS NOT
ELSEWHERE CLASSIFIED**

- 295.40 Schizophreniform disorder
- 298.80 Brief reactive psychosis
- 295.70 Schizoaffective disorder
- 298.90 Atypical psychosis

NEUROTIC DISORDERS: These are in-
cluded in Affective, Anxiety, Somato-
form, Dissociative, and Psychosexual
Disorders. In order to facilitate the
identification of the categories that in
DSM-II were grouped together in the
class of Neuroses, the DSM-II terms are
included separately in parentheses
after the corresponding categories.
These DSM-II terms are included in ICD-
9-CM and therefore are acceptable as
alternatives to the recommended DSM-
III terms that precede them.

AFFECTIVE DISORDERS**Major affective disorders**

Code major depressive episode in fifth
digit: 6 = in remission, 4 = with
psychotic features (the unofficial non-
ICD-9-CM fifth digit 7 may be used
instead to indicate that the psychotic
features are mood-incongruent), 3 =
with melancholia, 2 = without melan-
cholia, 0 = unspecified.

Code manic episode in fifth digit:
6 = in remission, 4 = with psychotic
features (the unofficial non-ICD-9-CM
fifth digit 7 may be used instead to in-
dicate that the psychotic features are
mood-incongruent), 2 = without psy-
chotic features, 0 = unspecified.

- Bipolar disorder,
- 296.6x mixed, _____
 - 296.4x manic, _____
 - 296.5x depressed, _____

- Major depression,
- 296.2x single episode, _____
 - 296.3x recurrent, _____

Other specific affective disorders

- 301.13 Cyclothymic disorder
- 300.40 Dysthymic disorder
(or Depressive neurosis)

Atypical affective disorders

- 296.70 Atypical bipolar disorder
- 296.82 Atypical depression

ANXIETY DISORDERS

Phobic disorders (or Phobic neuroses)

- 300.21 Agoraphobia with panic attacks
- 300.22 Agoraphobia without panic attacks
- 300.23 Social phobia
- 300.29 Simple phobia

Anxiety states (or Anxiety neuroses)

- 300.01 Panic disorder
- 300.02 Generalized anxiety disorder
- 300.30 Obsessive compulsive disorder
(or Obsessive compulsive neurosis)

Post-traumatic stress disorder

- 308.30 acute
- 309.81 chronic or delayed
- 300.00 Atypical anxiety disorder

SOMATOFORM DISORDERS

- 300.81 Somatization disorder
- 300.11 Conversion disorder
(or Hysterical neurosis, conversion type)
- 307.80 Psychogenic pain disorder
- 300.70 Hypochondriasis
(or Hypochondriacal neurosis)
- 300.70 Atypical somatoform disorder
(300.71)

**DISSOCIATIVE DISORDERS
(OR HYSTERICAL NEUROSES,
DISSOCIATIVE TYPE)**

- 300.12 Psychogenic amnesia
- 300.13 Psychogenic fugue
- 300.14 Multiple personality
- 300.60 Depersonalization disorder
(or Depersonalization neurosis)
- 300.15 Atypical dissociative disorder

PSYCHOSEXUAL DISORDERS**Gender identity disorders**

Indicate sexual history in the fifth digit of Transsexualism code: 1 = asexual, 2 = homosexual, 3 = heterosexual, 0 = unspecified.

- 302.5x Transsexualism, _____
- 302.60 Gender identity disorder of childhood
- 302.85 Atypical gender identity disorder

Paraphilias

- 302.81 Fetishism
- 302.30 Transvestism
- 302.10 Zoophilia
- 302.20 Pedophilia
- 302.40 Exhibitionism
- 302.82 Voyeurism
- 302.83 Sexual masochism
- 302.84 Sexual sadism
- 302.90 Atypical paraphilia

Psychosexual dysfunctions

- 302.71 Inhibited sexual desire
- 302.72 Inhibited sexual excitement
- 302.73 Inhibited female orgasm
- 302.74 Inhibited male orgasm
- 302.75 Premature ejaculation
- 302.76 Functional dyspareunia
- 306.51 Functional vaginismus
- 302.70 Atypical psychosexual dysfunction

Other psychosexual disorders

- 302.00 Ego-dystonic homosexuality
- 302.89 Psychosexual disorder not elsewhere classified

FACTITIOUS DISORDERS

- 300.16 Factitious disorder with psychological symptoms
- 301.51 Chronic factitious disorder with physical symptoms
- 300.19 Atypical factitious disorder with physical symptoms

**DISORDERS OF IMPULSE CONTROL
NOT ELSEWHERE CLASSIFIED**

- 312.31 Pathological gambling
- 312.32 Kleptomania
- 312.33 Pyromania
- 312.34 Intermittent explosive disorder
- 312.35 Isolated explosive disorder
- 312.39 Atypical impulse control disorder

ADJUSTMENT DISORDER

- 309.00 with depressed mood
- 309.24 with anxious mood
- 309.28 with mixed emotional features
- 309.30 with disturbance of conduct
- 309.40 with mixed disturbance of emotions and conduct
- 309.23 with work (or academic) inhibition
- 309.83 with withdrawal
- 309.90 with atypical features

PSYCHOLOGICAL FACTORS AFFECTING PHYSICAL CONDITION

- Specify physical condition on Axis III.
- 316.00 Psychological factors affecting physical condition

PERSONALITY DISORDERS

Note: These are coded on Axis II.

- 301.00 Paranoid
- 301.20 Schizoid
- 301.22 Schizotypal
- 301.50 Histrionic
- 301.81 Narcissistic
- 301.70 Antisocial
- 301.83 Borderline
- 301.82 Avoidant
- 301.60 Dependent
- 301.40 Compulsive
- 301.84 Passive-Aggressive
- 301.89 Atypical, mixed or other personality disorder

V CODES FOR CONDITIONS NOT ATTRIBUTABLE TO A MENTAL DISORDER THAT ARE A FOCUS OF ATTENTION OR TREATMENT

- V65.20 Malingering
- V62.89 Borderline intellectual functioning (V62.88)
- V71.01 Adult antisocial behavior
- V71.02 Childhood or adolescent antisocial behavior
- V62.30 Academic problem
- V62.20 Occupational problem
- V62.82 Uncomplicated bereavement
- V15.81 Noncompliance with medical treatment
- V62.89 Phase of life problem or other life circumstance problem
- V61.10 Marital problem
- V61.20 Parent-child problem
- V61.80 Other specified family circumstances
- V62.81 Other interpersonal problem

ADDITIONAL CODES

- 300.90 Unspecified mental disorder (nonpsychotic)
- V71.09 No diagnosis or condition on Axis I
- 799.90 Diagnosis or condition deferred on Axis I

- V71.09 No diagnosis on Axis II
- 799.90 Diagnosis deferred on Axis II

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Chapter Two

Use of This Manual

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Use of This Manual

MULTIAXIAL EVALUATION

A multiaxial evaluation requires that every case be assessed on each of several "axes," each of which refers to a different class of information. In order for the system to have maximal clinical usefulness, there must be a limited number of axes; there are five in the DSM-III multiaxial classification. The first three axes constitute the official diagnostic assessment.

Each individual is evaluated on each of these axes:

- Axis I** Clinical Syndromes
 Conditions Not Attributable to a Mental Disorder That Are a
 Focus of Attention or Treatment
 Additional Codes
- Axis II** Personality Disorders
 Specific Developmental Disorders
- Axis III** Physical Disorders and Conditions

Axes IV and V are available for use in special clinical and research settings and provide information supplementing the official DSM-III diagnoses (Axes I, II, and III) that may be useful for planning treatment and predicting outcome:

- Axis IV** Severity of Psychosocial Stressors
- Axis V** Highest Level of Adaptive Functioning Past Year

Use of the DSM-III multiaxial classification ensures that attention is given to certain types of disorders, aspects of the environment, and areas of functioning that might be overlooked if the focus were on assessing a single presenting problem.

Axes I and II

Axes I and II comprise the entire classification of mental disorders plus Conditions Not Attributable to a Mental Disorder That Are a Focus of Attention or Treatment. The disorders listed on Axis II are the Personality Disorders (for adults and, in some cases, for children and adolescents) and the Specific Developmental Disorders (for children and adolescents and, in some cases, for adults). The remaining disorders and conditions are included in Axis I. This separation ensures that consideration is given to the possible presence of disorders that are frequently overlooked when attention is directed to the usually more florid Axis I disorder.

In some instances an individual may have a disorder on both axes. For exam-

ple, an adult may have Major Depression noted on Axis I and Compulsive Personality Disorder on Axis II, or a child may have Conduct Disorder noted on Axis I and Developmental Language Disorder on Axis II. In other instances there may be no disorder on Axis I, the reason for seeking treatment being limited to a condition noted on Axis II. In this latter case, the clinician should write: *Axis I: V71.09 No diagnosis or condition on Axis I.* On the other hand, if a disorder is noted on Axis I but there is no evidence of an Axis II disorder, the clinician should write: *Axis II: V71.09 No diagnosis on Axis II, or one of the Conditions Not Attributable to a Mental Disorder should be recorded.*

Multiple diagnoses within Axes I and II

On both Axes I and II, multiple diagnoses should be made when necessary to describe the current condition. This applies particularly to Axis I, in which, for example, an individual may have both a Substance Use Disorder and an Affective Disorder. It is possible to have multiple diagnoses within the same class. For example, it is possible to have several Substance Use Disorders or, in the class of Affective Disorders, it is possible to have Major Depression superimposed on Dysthymic Disorder or Bipolar Disorder superimposed on Cyclothymic Disorder. In other classes, such as Schizophrenic Disorders, however, each of the subtypes is mutually exclusive.

Within Axis II, the diagnosis of multiple Specific Developmental Disorders is common. For some adults the persistence of a Specific Developmental Disorder and the presence of a Personality Disorder may require that both be noted on Axis II. Usually, a single Personality Disorder will be noted; but when the individual meets the criteria for more than one, all should be recorded.

Axis II and description of personality features

Axis II can be used to indicate specific personality traits when no Personality Disorder exists. For example, compulsive traits can be recorded on Axis II for an individual for whom Major Depression is noted on Axis I. Even when a Personality Disorder is noted on Axis II, the clinician may wish to indicate other personality characteristics—e.g., paranoid traits can be noted on Axis II for an individual who is also described as having Compulsive Personality Disorder on this same axis. (Code numbers should not be used when personality *traits* are noted, since a code number indicates a Personality *Disorder*.)

Principal diagnosis

When an individual receives more than one diagnosis, the *principal* diagnosis is the condition that was chiefly responsible for occasioning the evaluation or admission to clinical care. In most cases this condition will be the main focus of attention or treatment. The principal diagnosis may be an Axis I or an Axis II diagnosis; but when an Axis II diagnosis is the principal diagnosis the notation should be followed by the phrase “(Principal diagnosis).”

Example: Axis I: 303.93 Alcohol Dependence, In Remission
Axis II: 301.70 Antisocial Personality Disorder (Principal diagnosis)

When an individual has both an Axis I and an Axis II diagnosis, the principal diagnosis will be assumed to be on Axis I unless the Axis II diagnosis is followed by the qualifying phrase "(Principal diagnosis)."

When multiple diagnoses are made on either Axis I or Axis II, they should be listed within each axis in the order of focus of attention or treatment. For example, if an individual with Schizophrenia, Paranoid Type, Chronic, comes to an emergency room for treatment of Alcohol Intoxication, the diagnosis should be listed:

Axis I: 303.00 Alcohol Intoxication
 295.32 Schizophrenia, Paranoid Type, Chronic

Provisional diagnosis

In some instances not enough information will be available to make a firm diagnosis. The clinician may wish to indicate a significant degree of diagnostic uncertainty by writing "(Provisional)" following the diagnosis—e.g., Schizophreniform Disorder (Provisional, rule out Organic Delusional Syndrome).

Levels of diagnostic certainty

Frequently a diagnostic evaluation yields insufficient information to make a specific diagnosis. The following table indicates the various ways in which a clinician may indicate diagnostic uncertainty:

Term	Examples of clinical situations
V Codes (for Conditions Not Attributable to a Mental Disorder That Are a Focus of Attention or Treatment)	Insufficient information to know whether or not a presenting problem is attributable to a mental disorder, e.g., Academic Problem; Adult Antisocial Behavior.
799.90 Diagnosis or Condition Deferred on Axis I	Information inadequate to make any diagnostic judgment about an Axis I diagnosis or condition.
799.90 Diagnosis Deferred on Axis II	Same for an Axis II diagnosis.
300.90 Unspecified Mental Disorder (non-psychotic)	Enough information available to rule out a psychotic disorder, but further specification is not possible.
298.90 Atypical Psychosis	Enough information available to determine the presence of a psychotic disorder, but further specification is not possible.

Term	Examples of clinical situations
Atypical (class of disorder)	Enough information available to indicate the class of disorder that is present, but further specification is not possible, because either there is not sufficient information to make a more specific diagnosis, or the clinical features of the disorder do not meet the criteria for any of the other categories, e.g., Atypical Affective Disorder.
Specific diagnosis (Provisional)	Enough information available to make a "working" diagnosis, but the clinician wishes to indicate a significant degree of diagnostic uncertainty, e.g., Schizophreniform Disorder (Provisional).

Axis III. Physical Disorders or Conditions

Axis III permits the clinician to indicate any current physical disorder or condition that is potentially relevant to the understanding or management of the individual. These are the conditions outside of the mental disorders section of ICD-9-CM. In some instances the condition may be etiologically significant (e.g., a neurologic disorder associated with Dementia); in other instances the physical disorder may not be etiologic, but important in the overall management of the individual (e.g., diabetes in a child with a Conduct Disorder). In yet other instances, the clinician may wish to note significant associated physical findings, such as "soft neurological signs." Multiple diagnoses are permitted.

Axis IV. Severity of Psychosocial Stressors

Axis IV provides a coding of the overall severity of a stressor judged to have been a significant contributor to the development or exacerbation of the current disorder. An individual's prognosis may be better when a disorder develops as a consequence of a severe stressor than when it develops after no stressor or a minimal stressor.

Rating the severity of the stressor. This rating should be based on the clinician's assessment of the stress as an "average" person in similar circumstances and with similar sociocultural values would experience from the particular psychosocial stressor(s). This judgment involves consideration of the following: the amount of change in the individual's life caused by the stressor, the degree to which the event is desired and under the individual's control, and the number of stressors. Even though a specific stressor may have greater impact on an individual who is especially vulnerable or has certain internal conflicts, the rating should be based on the severity of the stressor itself, not on the individual's vulnerability to the particular stressor. If a vulnerability to stress exists, it will frequently be due to a mental disorder that is coded on Axis I or II.

In most instances the psychosocial stressor will have occurred within a year prior to the current disorder (Post-traumatic Stress Disorder is a notable exception). In some instances the stressor is the anticipation of a future event: for example, the knowledge that one will soon retire. Although a stressor frequently

plays a precipitating role in a disorder, it may also be a consequence of the individual's psychopathology—e.g., Alcohol Dependence may lead to marital problems and divorce, which can then become stressors contributing to the development of a Major Depression. The current disorder that is related to the psychosocial stressor may be either a clinical syndrome, coded on Axis I, or an exacerbation of a Personality or Specific Developmental Disorder, coded on Axis II.

In addition to the severity rating, in certain settings it may be useful to note the specific psychosocial stressor (e.g., chronic marital discord about sharing household duties). This information may be important in formulating a treatment plan that includes attempts to remove the psychosocial stressor or to help the individual cope with it. More than one psychosocial stressor may be judged etiologically significant by the clinician, but rarely will more than four be recorded. The stressors should be noted as specifically as possible and listed in order of their importance.

The severity rating should reflect the summed effect of all of the psychosocial stressors that are listed. The following codes and terms may be used as guides in making the rating:

Code	Term	Adult examples	Child or adolescent examples
1	None	No apparent psychosocial stressor	No apparent psychosocial stressor
2	Minimal	Minor violation of the law; small bank loan	Vacation with family
3	Mild	Argument with neighbor; change in work hours	Change in schoolteacher; new school year
4	Moderate	New career; death of close friend; pregnancy	Chronic parental fighting; change to new school; illness of close relative; birth of sibling
5	Severe	Serious illness in self or family; major financial loss; marital separation; birth of child	Death of peer; divorce of parents; arrest; hospitalization; persistent and harsh parental discipline
6	Extreme	Death of close relative; divorce	Death of parent or sibling; repeated physical or sexual abuse
7	Catastrophic	Concentration camp experience; devastating natural disaster	Multiple family deaths
0	Unspecified	No information, or not applicable	No information, or not applicable

Types of psychosocial stressors to be considered. To ascertain etiologically significant psychosocial stressors, the following areas may be considered:

Conjugal (marital and nonmarital): e.g., engagement, marriage, discord, separation, death of spouse.

Parenting: e.g., becoming a parent, friction with child, illness of child.

Other interpersonal: problems with one's friends, neighbors, associates, or non-conjugal family members, e.g., illness of best friend, discordant relationship with boss.

Occupational: includes work, school, homemaker, e.g., unemployment, retirement, school problems.

Living circumstances: e.g., change in residence, threat to personal safety, immigration.

Financial: e.g., inadequate finances, change in financial status.

Legal: e.g., arrested, jailed, lawsuit or trial.

Developmental: phases of the life cycle, e.g., puberty, transition to adult status, menopause, "becoming 50."

Physical illness or injury: e.g., illness, accident, surgery, abortion. (Note: A physical disorder is listed on Axis III whenever it is related to the development or management of an Axis I or II disorder. A physical disorder can also be a psychosocial stressor if its impact is due to its meaning to the individual, in which case it would be listed on both Axis III and Axis IV.)

Other psychosocial stressors: e.g., natural or manmade disaster, persecution, unwanted pregnancy, out-of-wedlock birth, rape.

Family factors (children and adolescents): In addition to the above, for children and adolescents the following stressors may be considered: cold or distant relationship between parents; overtly hostile relationship between parents; physical or mental disturbance in family members; cold or distant parental behavior toward child; overtly hostile parental behavior toward child; parental intrusiveness; inconsistent parental control; insufficient parental control; insufficient social or cognitive stimulation; anomalous family situation, e.g., single parent, foster family; institutional rearing; loss of nuclear family members.

Axis V. Highest Level of Adaptive Functioning Past Year

Axis V permits the clinician to indicate his or her judgment of an individual's highest level of adaptive functioning (for at least a few months) during the past year. This information frequently has prognostic significance, because usually an individual returns to his or her previous level of adaptive functioning after an episode of illness.

As conceptualized here, adaptive functioning is a composite of three major areas: social relations, occupational functioning, and use of leisure time. These three areas are to be considered together, although there is evidence that social relations should be given greater weight because of their particularly great prognostic significance. An assessment of the use of leisure time will affect the overall judgment only when there is no significant impairment in social relations and occupational functioning or when occupational opportunities are limited or absent (e.g., the individual is retired or handicapped).

Social relations include all relations with people, with particular emphasis on family and friends. The breadth and quality of interpersonal relationships should be considered.

Occupational functioning refers to functioning as a worker, student, or homemaker. The amount, complexity, and quality of the work accomplished

should be considered. The highest levels of adaptive functioning should be used only when high occupational productivity is not associated with a high level of subjective discomfort.

Use of leisure time includes recreational activities or hobbies. The range and depth of involvement and the pleasure derived should be considered.

The level noted should be descriptive of the individual's functioning regardless of whether or not special circumstances, such as concurrent treatment, may have been necessary to sustain that level.

Levels	Adult examples	Child or adolescent examples
1 SUPERIOR —Unusually effective functioning in social relations, occupational functioning, and use of leisure time.	Single parent living in deteriorating neighborhood takes excellent care of children and home, has warm relations with friends, and finds time for pursuit of hobby.	A 12-year-old girl gets superior grades in school, is extremely popular among her peers, and excels in many sports. She does all of this with apparent ease and comfort.
2 VERY GOOD —Better than average functioning in social relations, occupational functioning, and use of leisure time.	A 65-year-old retired widower does some volunteer work, often sees old friends, and pursues hobbies.	An adolescent boy gets excellent grades, works part-time, has several close friends, and plays banjo in a jazz band. He admits to some distress in "keeping up with everything."
3 GOOD —No more than slight impairment in either social or occupational functioning.	A woman with many friends functions extremely well at a difficult job, but says "the strain is too much."	An 8-year-old boy does well in school, has several friends, but bullies younger children.
4 FAIR —Moderate impairment in either social relations or occupational functioning, or some impairment in both.	A lawyer has trouble carrying through assignments; has several acquaintances, but hardly any close friends.	A 10-year-old girl does poorly in school, but has adequate peer and family relations.
5 POOR —Marked impairment in either social relations or occupational functioning, or moderate impairment in both.	A man with one or two friends has trouble keeping a job for more than a few weeks.	A 14-year-old boy almost fails in school and has trouble getting along with his peers.

Levels	Adult examples	Child or adolescent examples
6 VERY POOR— Marked impairment in both social relations and occupational functioning.	A woman is unable to do any of her housework and has violent outbursts toward family and neighbors.	A 6-year-old girl's needs special help in all subjects and has virtually no peer relationships.
7 GROSSLY IMPAIRED —Gross impairment in virtually all areas of functioning.	An elderly man needs supervision to maintain minimal personal hygiene and is usually incoherent.	A 4-year-old boy needs constant restraint to avoid hurting himself and is almost totally lacking in skills.
0 UNSPECIFIED	No information.	No information.

**Examples of How To Record the Results
of a DSM-III Multiaxial Evaluation**

EXAMPLE 1

- Axis I:** 296.23 Major Depression, Single Episode, with Melancholia
303.93 Alcohol Dependence, In Remission
- Axis II:** 301.60 Dependent Personality Disorder (Provisional, rule out Borderline Personality Disorder)
- Axis III:** Alcoholic cirrhosis of liver
- Axis IV:** Psychosocial stressors: anticipated retirement and change in residence with loss of contact with friends
Severity: 4—Moderate
- Axis V:** Highest level of adaptive functioning past year: 3—Good

EXAMPLE 2

- Axis I:** 304.03 Heroin Dependence, In Remission
- Axis II:** 301.70 Antisocial Personality Disorder (Principal diagnosis); prominent paranoid traits
- Axis III:** None
- Axis IV:** Psychosocial stressors: No information
Severity: 0—Unspecified
- Axis V:** Highest level of adaptive functioning past year: 5—Poor

EXAMPLE 3

- Axis I:** 295.92 Schizophrenia, Undifferentiated Type, Chronic
V62.89 Borderline Intellectual Functioning (Provisional)
- Axis II:** V71.08 No diagnosis on Axis II

- Axis III:** Late effects of viral encephalitis
- Axis IV:** Psychosocial stressors: death of mother
Severity: 6—Extreme
- Axis V:** Highest level of adaptive functioning past year: 6—
Very poor

DIAGNOSTIC CRITERIA

Diagnostic criteria appear at the end of the text describing each specific diagnosis.

These criteria are offered as useful guides for making the diagnosis, since it has been demonstrated that the use of such criteria enhances diagnostic agreement among clinicians. It should be understood, however, that for most of the categories the criteria are based on clinical judgment, and have not yet been fully validated; with further experience and study, the criteria will, in many cases, undoubtedly be revised.

Designation by capital letters indicates multiple criteria the presence of *all* of which constitutes the guide to making the diagnosis.

TYPES OF INFORMATION IN THE TEXT

In order to ensure consistency and comprehensiveness in the descriptions of the disorders, information has been included under each of the following headings. In some instances, when many of the specific disorders, such as Substance Use Disorders, share common features, this information is included in the introduction to the entire section.

The first paragraph states the **essential features** of the disorder. These are the features that are generally required to make the diagnosis and that are always present.

Associated features. Features that are often, but not invariably, present.

Age at onset. The age when the disorder usually becomes apparent.

Course. The usual course or outcome of the disorder.

Impairment. Conceptualized primarily as impairment in social and occupational functioning.

Complications. Disorders or events (e.g., suicide) that may develop as a result of the disorder. In some cases the distinction among complications, impairment, and associated features is arbitrary.

Predisposing factors. Characteristics of an individual that can be identified before the development of the disorder and that place him or her at higher risk for developing the disorder. Not included in this section are general societal or environmental conditions (such as poverty) that may predispose all individuals exposed to these conditions to develop the disorder.

Prevalence. The prevalence of the disorder is often expressed as the proportion of adults who at some time in their lives met the criteria for the disorder. This method of presentation has the advantage of being readily understandable, but it is highly dependent on the age at onset and the relative proportion of individuals in the population who have reached that age. The data are often presented as a range, based on more than a single study.

When data from epidemiological studies are not available, the prevalence is stated in general terms preceded by the word “apparently” to indicate that the judgment is based on clinical experience. The term “apparently rare” is applied to disorders that may not be seen by a clinician in many years of practice.

Sex ratio. The relative frequency with which the disorder is diagnosed in men and women.

Familial pattern. Indication of whether the disorder is more common among biologically related family members than in the general population. When this is the case, it does not necessarily indicate a genetic mechanism. Unless positive information has been replicated in several studies, “No information” is noted.

Differential diagnosis. Disorders that should be distinguished from the one being presented are discussed, generally in the order in which they appear in the classification.

EXPLANATION OF TERMS AND CONVENTIONS

Atypical. This term is used to indicate a category within a class of disorders that is residual to the specific categories in that class, although it is recognized that in some settings what is regarded as an atypical disorder may actually be more common than any of the specific disorders in that particular class. (In the literature the term “atypical” has sometimes been used in a different sense—to describe a specific diagnostic category that has unusual features.)

Physical disorders. The term “physical disorders” is used to refer to any disorder listed in ICD-9-CM outside the chapter on mental disorders.

Terms in parentheses. In order to facilitate the identification of the categories that in DSM-II were grouped together in the class of Neuroses, the DSM-II terms are included separately in parentheses after the corresponding categories. These DSM-II terms are included in ICD-9-CM and therefore are acceptable as alternatives to the recommended DSM-III terms that precede them.

Not due to another disorder. This phrase is used to indicate that the disorder being described is not diagnosed if the disturbance is apparently symptomatic of another disorder. For example, in the diagnostic criteria for Schizophrenia, there is the phrase, “Not due to any Organic Mental Disorder.” This means that the diagnosis of Schizophrenia is not given if the characteristic symptoms, such as delusions or hallucinations, are caused by an Organic Mental Disorder.

Chapter Three

The Diagnostic Categories: Text and Criteria

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Disorders Usually First Evident In Infancy, Childhood, Or Adolescence

The disorders described in this chapter are those that usually arise and are first evident in infancy, childhood, or adolescence. (Exceptions are the Gender Identity Disorders, classified with the other Psychosexual Disorders.) There is no arbitrary age limit here that defines childhood and adolescence, and this section includes some disorders characteristic of older adolescents, such as Identity Disorder, which may first appear in early adulthood.

In diagnosing an infant, child, or adolescent, the clinician should first consider the diagnoses included in this section. If an appropriate diagnosis cannot be found, disorders described elsewhere in this manual should be considered.

Because the *essential* features of Affective Disorders and Schizophrenia are the same in children and adults, there are no special categories corresponding to these disorders in this section of the classification. For example, if a child or adolescent has an illness that meets the criteria for Major Depression, Dysthymic Disorder, or Schizophrenia, these diagnoses should be given, regardless of the age of the individual. (In some instances, age-specific *associated* features that apply to infants, children, or adolescents are included in the text.)

Other diagnostic categories that often will be appropriate for children or adolescents are the following:

Organic Mental Disorders
Substance Use Disorders
Schizophrenic Disorders
Affective Disorders
Schizophreniform Disorder
Anxiety Disorders

Somatoform Disorders
Personality Disorders*
Psychosexual Disorders
Adjustment Disorder
Psychological Factors Affecting
Physical Condition

Adults should be given diagnoses from this section if, as infants, children, or adolescents, they manifested any of these conditions and if the condition has persisted. Examples include Attention Deficit Disorder, Residual Type, and some cases of Conduct Disorder. Finally, some individuals may develop in adulthood a disorder, such as Anorexia Nervosa, that is included in this section because the disorder *usually* develops in children or adolescents.

The classes of disorders described in this section can be separated into five major groups on the basis of the predominant area of disturbance. This subgrouping is done for heuristic purposes, and it is recognized that the designation of the area of predominant disturbance is at best an approximation.

I. Intellectual Mental Retardation

* For a discussion of the diagnosis of Personality Disorders in children and adolescents, see p. 305.

- II. Behavioral (overt)
 - Attention Deficit Disorder
 - Conduct Disorder
- III. Emotional
 - Anxiety Disorders of Childhood or Adolescence
 - Other Disorders of Infancy, Childhood, or Adolescence
- IV. Physical
 - Eating Disorders
 - Stereotyped Movement Disorders
 - Other Disorders with Physical Manifestations
- V. Developmental
 - Pervasive Developmental Disorders
 - Specific Developmental Disorders

Children often have problems in development that are not subsumed within the specific DSM-III diagnostic categories, such as precocious sexual activity and aggressive behavior. In these cases a diagnosis of Unspecified Mental Disorder may be used, and the predominant features should be described. Similarly, many children have problems that do not warrant diagnosis as a mental disorder. Such conditions can be noted with a V code, such as Parent-Child Problem, Childhood or Adolescent Antisocial Behavior, or Other Specified Family Circumstances.

A category for child abuse is not included in DSM-III as a mental disorder (of the abusing parent) since child abuse as an act can be a symptom of many different disorders or not be associated with any mental disorder. Child abuse is not included as a separate V code because, when not due to a mental disorder, it represents merely one of many types of family circumstances that can be coded as V61.80, Other Specified Family Circumstances. (In ICD-9-CM, there is a special code, V61.21, Child Abuse, that refers to the situation of child abuse involving child, parent or both. The code 995.5, Child Maltreatment Syndrome, identifies the abused child who presents with injuries or other trauma.)

MENTAL RETARDATION*

The essential features are: (1) significantly subaverage general intellectual functioning, (2) resulting in, or associated with, deficits or impairments in adaptive behavior, (3) with onset before the age of 18. The diagnosis is made regardless of whether or not there is a coexisting mental or physical disorder.

General intellectual functioning is defined as an intelligence quotient (IQ) obtained by assessment with one or more of the individually administered general intelligence tests. Significantly subaverage intellectual functioning is defined as an IQ of 70 or below on an individually administered IQ test. Since any measurement is fallible, an IQ score is generally thought to involve an error of measurement of approximately five points; hence, an IQ of 70 is considered to represent a band or zone of 65 to 75. Treating the IQ with some flexibility permits the inclusion in the Mental Retardation category of individuals with IQs some-

* The definitions of Mental Retardation were written in accordance with the terminology and classification of the American Association on Mental Deficiency.

what higher than 70 who truly need special education or other programs. It also permits exclusion from the diagnosis of those with IQs somewhat lower than 70 if the clinical judgment is that there are no significant deficits or impairment in adaptive functioning.

Adaptive behavior refers to the effectiveness with which an individual meets the standards of personal independence and social responsibility expected of his or her age and cultural group. There are scales designed to quantify adaptive behavior, but none is considered sufficiently reliable and valid to be used alone to evaluate this aspect of functioning. Therefore, clinical judgment is necessary for the assessment of general adaptation, the individual's age being taken into consideration. The IQ level of 70 was chosen as the upper limit for Mental Retardation because most people with IQs below 70 are so limited in their adaptive functioning that they require special services and protection, particularly during the school-age years.

The arbitrary IQ ceiling values are based on data indicating a positive association between intelligence (as measured by IQ score) and adaptive behavior. This association declines at the upper levels of Mild Retardation. Some individuals with an IQ near but below 70 may not have the impairment in adaptive behavior required for a diagnosis of Mental Retardation.

When the clinical picture develops for the first time after the age of 18, the syndrome is a Dementia, not Mental Retardation, and is coded within the Organic Mental Disorders section of the classification (p. 162). When the clinical picture develops before the age of 18 in an individual who previously had normal intelligence, Mental Retardation and Dementia should *both* be diagnosed.

Etiologic factors may be primarily biological, psychosocial, or an interaction of both. When a known biological factor is present, the specific biological condition should be noted on Axis III.

Associated features. The prevalence of other mental disorders, such as Stereotyped Movement Disorder, Infantile Autism, and Attention Deficit Disorder with Hyperactivity, is three or four times greater among children with Mental Retardation than in the general population. When another mental disorder is present, it should also be coded on Axis I.

There may be no associated behavioral features other than those reflecting the Mental Retardation itself, as in some cases of Down's syndrome, or there may be other behavioral concomitants, such as irritability, aggressivity, temper tantrums, or stereotyped movements. (When these other behavioral symptoms require attention or treatment, their presence may be noted in the fifth digit, p. 41).

Often there are multiple neurological abnormalities, involving neuromuscular function, vision or hearing, or seizures, particularly among individuals with Severe Mental Retardation. These should be noted on Axis III.

Course. When a specific biological abnormality is present, the course is usually chronic and without remission, and without treatment the disorder may become more severe. In mild forms of the disorder with no known etiology, the course may be self-limited as the individual experiences an increase in intellec-

tual functioning (e.g., from a more stimulating environment) or displays more adaptive behavior (e.g., outside of the demanding environment of school to which he or she was unable to adapt).

Impairment. By definition, there are always deficits or impairments in adaptive functioning. The degree of impairment is correlated with the level of general intellectual functioning and the presence of the associated features noted above.

Complications. The major complication is inability to function independently and hence a continuing need for supervision and financial support.

Etiologic factors and familial pattern. Etiologic factors may be primarily biological, psychosocial, or an interaction of both. In 25% of the cases, the etiologic factors are known biological abnormalities, the most common being chromosomal and metabolic disorders such as Down's syndrome and phenylketonuria. In such cases the diagnosis is usually established at birth or at a relatively young age, and the severity of the Mental Retardation is generally moderate to profound. Heavy maternal alcohol consumption during pregnancy can cause the fetal alcohol syndrome, manifested by retarded growth, various craniofacial and limb anomalies, and Mental Retardation.

Mental Retardation due to known biological factors is as likely to occur among children of upper socioeconomic classes as among those of lower socioeconomic classes. In such cases there is no increased prevalence of Mental Retardation in other family members unless the underlying biological condition is a genetically determined disorder, such as phenylketonuria or Tay-Sachs disease.

In the remaining 75% of the cases, no known specific biological factor accounts for the disorder; the level of intellectual impairment is usually mild, with IQs between 50 and 70; and the diagnosis is commonly not made until school entrance. The lower socioeconomic classes are overrepresented in these cases of Mental Retardation; the significance of this is not clear. There is often a familial pattern of similar degrees of severity of Mental Retardation in parents and siblings.

Mental Retardation without known biological etiology may be associated with psychosocial deprivation of various types, such as deprivation of social, linguistic, and intellectual stimulation. However, the specific etiology of these forms of Mental Retardation is unknown. Three sets of etiologic factors are probably involved, either singly or in combination: genetic factors, environmental biological factors such as malnutrition, and early child-rearing experiences.

Prevalence. At any one point in time, approximately 1% of the population meets the criteria for Mental Retardation.

Sex ratio. The disorder is nearly twice as common among males as among females.

Subtypes. There are four subtypes, reflecting the degree of intellectual im-

pairment and designated as Mild, Moderate, Severe, and Profound. IQ levels to be used as guides for distinguishing the four subtypes are given below:

Subtypes of Mental	
Retardation	IQ Levels
Mild	50-70
Moderate	35-49
Severe	20-34
Profound	Below 20

317.0(x) Mild Mental Retardation

Mild Mental Retardation is roughly equivalent to the educational category "educable." This group makes up the largest segment of those with the disorder—about 80%. Individuals with this level of Mental Retardation can develop social and communication skills during the preschool period (ages 0-5), have minimal impairment in sensorimotor areas, and often are not distinguishable from normal children until a later age. By their late teens they can learn academic skills up to approximately the sixth-grade level; and during the adult years, they can usually achieve social and vocational skills adequate for minimum self-support, but may need guidance and assistance when under unusual social or economic stress.

318.0(x) Moderate Mental Retardation

Moderate Mental Retardation is roughly equivalent to the educational category of "trainable." This group makes up 12% of the entire population of individuals with Mental Retardation. Those with this level of Mental Retardation during the preschool period can talk or learn to communicate, but they have only poor awareness of social conventions. They may profit from vocational training and can take care of themselves with moderate supervision. During the school-age period, they can profit from training in social and occupational skills, but are unlikely to progress beyond the second-grade level in academic subjects. They may learn to travel alone in familiar places. During their adult years they may be able to contribute to their own support by performing unskilled or semi-skilled work under close supervision in sheltered workshops. They need supervision and guidance when under mild social or economic stress.

318.1(x) Severe Mental Retardation

This group makes up 7% of individuals with Mental Retardation. During the preschool period there is evidence of poor motor development and minimal speech, and they develop little or no communicative speech. During the school-age period, they may learn to talk and can be trained in elementary hygiene skills. They are generally unable to profit from vocational training. During their adult years they may be able to perform simple work tasks under close supervision.

318.2(x) Profound Mental Retardation

This group constitutes less than 1% of individuals with Mental Retardation. During the preschool period these children display minimal capacity for sensori-

motor functioning. A highly structured environment, with constant aid and supervision, is required. During the school-age period, some further motor development may occur and the children may respond to minimal or limited training in self-care. Some speech and further motor development may take place during the adult years, and very limited self-care may be possible, in a highly structured environment with constant aid and supervision.

319.0(x) Unspecified Mental Retardation

This category should be used when there is a strong presumption of Mental Retardation but the individual is untestable by standard intelligence tests. This may be the case when children, adolescents or adults are too impaired or uncooperative to be tested. In the case of infants, since the available tests, such as the Bayley, Cattell, and others, do not yield numerical IQ values, this may be the case when there is a clinical judgment of significant subaverage intellectual functioning. In general, the younger the age, the more difficult it is to make a diagnosis of Mental Retardation, except for those with profound impairment.

This category should not be used when the intellectual level is presumed to be above 70 (see V code for Borderline Intellectual Functioning, p. 332).

Differential diagnosis. The diagnosis of Mental Retardation should be made whenever present regardless of the presence of another diagnosis. In **Specific Developmental Disorders** there is a delay or failure of development in a specific area, such as reading or language, but in other areas of development the child is developing normally. In contrast, the child with Mental Retardation shows general delays in development in many areas. In **Pervasive Developmental Disorders** there are distortions in the timing, rate, and sequence of many basic psychological functions involved in the development of social skills and language. Furthermore, there are severe qualitative abnormalities that are not normal for any stage of development, whereas in Mental Retardation there are generalized delays in development, but the children behave as if they were passing through an earlier normal developmental stage. Mental Retardation may, however, coexist with Specific Developmental Disorders, and frequently coexists with Pervasive Developmental Disorders.

The V code **Borderline Intellectual Functioning** is given when there are deficits in adaptive behavior associated with borderline intellectual functioning, which generally is in the IQ range of 71 to 84. Differentiating Mild Mental Retardation from Borderline Intellectual Functioning requires careful consideration of all available information, including psychological test scores.

Diagnostic criteria for Mental Retardation

A. Significantly subaverage general intellectual functioning: an IQ of 70 or below on an individually administered IQ test (for infants, since available intelligence tests do not yield numerical values, a clinical judgment of significant subaverage intellectual functioning).

B. Concurrent deficits or impairments in adaptive behavior, taking the person's age into consideration.

C. Onset before the age of 18.

(If there are behavioral symptoms requiring attention or treatment [e.g., aggressive behavior, self-mutilation, anxiety symptoms] that are not part of another disorder, the non-ICD-9-CM code "1" may be recorded in the fifth digit. Otherwise, code "0".)

ATTENTION DEFICIT DISORDER

The essential features are signs of developmentally inappropriate inattention and impulsivity. In the past a variety of names have been attached to this disorder, including: Hyperkinetic Reaction of Childhood, Hyperkinetic Syndrome, Hyperactive Child Syndrome, Minimal Brain Damage, Minimal Brain Dysfunction, Minimal Cerebral Dysfunction, and Minor Cerebral Dysfunction. In this manual Attention Deficit is the name given to this disorder, since attentional difficulties are prominent and virtually always present among children with these diagnoses. In addition, though excess motor activity frequently diminishes in adolescence, in children who have the disorder, difficulties in attention often persist.

There are two subtypes of the active disorder, Attention Deficit Disorder with Hyperactivity, and Attention Deficit Disorder without Hyperactivity, although it is not known whether they are two forms of a single disorder or represent two distinct disorders. Finally, there is a residual subtype for individuals once diagnosed as having Attention Deficit Disorder with Hyperactivity in which hyperactivity is no longer present, but other signs of the disorder persist.

314.01 Attention Deficit Disorder with Hyperactivity

The essential features are signs of developmentally inappropriate inattention, impulsivity, and hyperactivity. In the classroom, attentional difficulties and impulsivity are evidenced by the child's not staying with tasks and having difficulty organizing and completing work. The children often give the impression that they are not listening or that they have not heard what they have been told. Their work is sloppy and is performed in an impulsive fashion. On individually administered tests, careless, impulsive errors are often present. Performance may be characterized by oversights, such as omissions or insertions, or misinterpretations of easy items even when the child is well motivated, not just in situations that hold little intrinsic interest. Group situations are particularly difficult for the child, and attentional difficulties are exaggerated when the child is in the classroom, where sustained attention is expected.

At home, attentional problems are shown by a failure to follow through on parental requests and instructions and by the inability to stick to activities, including play, for periods of time appropriate for the child's age.

Hyperactivity in young children is manifested by gross motor activity, such as excessive running or climbing. The child is often described as being on the go, "running like a motor," and having difficulty sitting still. Older children and adolescents may be extremely restless and fidgety. Often it is the quality of the motor behavior that distinguishes this disorder from ordinary overactivity in that hyperactivity tends to be haphazard, poorly organized, and not goal-directed.

In situations in which high levels of motor activity are expected and appro-